

AMCARDIA* TABLET 5mg/10mg
(Amlodipine Tablets 5mg/10 mg)

Name and strength of Active ingredients

Each uncoated tablet contains:

Amlodipine Besilate BP equivalent to Amlodipine 5 mg / 10 mg

Product Description:

Amcardia Tablet 5 mg: White to off-white round flat tablets with break line on one side.

Amcardia Tablet 10 mg: White to off-white round flat tablets with break line on one side.

Dosage Form:

Tablets

Pharmacodynamics:

Mechanism of Action:

Amcardia is a dihydropyridine calcium antagonist (calcium ion antagonist or slow-channel blocker) that inhibits the transmembrane influx of calcium ions into vascular smooth muscle and cardiac muscle. Experimental data suggest that AMCARDIA binds to both dihydropyridine and nondihydropyridine binding sites. The contractile processes of cardiac 2 muscle and vascular smooth muscle are dependent upon the movement of extracellular calcium ions into these cells through specific ion channels.

AMCARDIA inhibits calcium ion influx across cell membranes selectively, with a greater effect on vascular smooth muscle cells than on cardiac muscle cells. Negative inotropic effects can be detected in vitro but such effects have not been seen in intact animals at therapeutic doses. AMCARDIA is a peripheral arterial vasodilator that acts directly on vascular smooth muscle to cause a reduction in peripheral vascular resistance and reduction in blood pressure.

The precise mechanisms by which AMCARDIA relieves angina have not been fully delineated, but are thought to include the following:

Exertional Angina: In patients with exertional angina, AMCARDIA reduces the total peripheral resistance (afterload) against which the heart works and reduces the rate pressure product, and thus myocardial oxygen demand, at any given level of exercise. Vasospastic Angina: AMCARDIA has been demonstrated to block constriction and restore blood flow in coronary arteries and arterioles in response to calcium, potassium epinephrine, serotonin, and thromboxane A2 analog in experimental animal models and in human coronary vessels in vitro. This inhibition of coronary spasm is responsible for the effectiveness of AMCARDIA in vasospastic (Prinzmetal's or variant) angina.

Pharmacokinetics:

Pharmacokinetics and Metabolism:

After oral administration of therapeutic doses of Amlodipine besilate, absorption produces peak plasma concentrations between 6 and 12 hours. Absolute bioavailability has been estimated to be between 64 and 90%. The bioavailability of AMCARDIA is not altered by the presence of food.

Amlodipine besilate is extensively (about 90%) converted to inactive metabolites via hepatic metabolism with 10% of the parent compound and 60% of the metabolites excreted in the urine. Elimination from the plasma is biphasic with a terminal elimination half-life of about 30-50 hours. Steady-state plasma levels of Amlodipine besilate are reached after 7 to 8 days of consecutive daily dosing.

The pharmacokinetics of AMCARDIA are not significantly influenced by renal impairment. Patients with renal failure may therefore receive the usual initial dose. Elderly patients and patients with hepatic insufficiency have decreased clearance of amlodipine with a resulting increase in AUC of approximately 40-60%, and a lower initial dose may be required. A similar increase in AUC was observed in patients with moderate to severe heart failure.

Indications:

Hypertension - AMCARDIA is indicated for the treatment of hypertension. It may be used alone or in combination with other antihypertensive agents.

Chronic Stable Angina - AMCARDIA is indicated for the treatment of chronic stable angina. AMCARDIA may be used alone or in combination with other antianginal agents. Vasospastic Angina (Prinzmetal's or Variant Angina) AMCARDIA is indicated for the treatment of confirmed or suspected vasospastic angina. AMCARDIA may be used as monotherapy or in combination with other antianginal drugs.

Recommended Dose:

For both hypertension and angina the usual initial dose is 5mg amlodipine once daily which may be increased to a maximum dose of 10mg depending on the individual patient's response.

No dose adjustment of amlodipine is required upon concomitant administration of thiazide diuretics, beta-blockers, and angiotensin-converting enzyme inhibitors.

Use in the Elderly: A normal dosage regimen is recommended. Amlodipine, used at similar doses in elderly or younger patients, is equally well tolerated.

Use in Children: Safety and effectiveness of amlodipine in children have not been established.

Use in Patients with Impaired Hepatic Function: As with all calcium antagonists, amlodipine half-life is prolonged in patients with impaired liver function and dosage recommendations have not been established. The drug should therefore be administered with caution in these patients.

Use in Renal Failure: Amlodipine may be used in such patients at normal doses. Changes in amlodipine plasma concentrations are not correlated with degree of renal impairment. Amlodipine is not dialyzable.

Route of Administration:

Oral

Contraindications:

Amlodipine is contraindicated in patients with a known sensitivity to dihydropyridines,

amlodipine, or any of the inert ingredients.'

Warning and Precautions:

WARNINGS

Increased Angina and/or Myocardial Infarction:

Rarely, patients, particularly those with severe obstructive coronary artery disease, have developed documented increased frequency, duration and/or severity of angina or acute myocardial infarction on starting calcium channel blocker therapy or at the time of dosage increase. The mechanism of this effect has not been elucidated.

PRECAUTIONS

General: Since the vasodilation induced by AMCARDIA is gradual in onset, acute hypotension has rarely been reported after oral administration of AMCARDIA. Nonetheless, caution should be exercised when administering AMCARDIA as with any other peripheral vasodilator particularly in patients with severe aortic stenosis.

'Use in Patients with Heart Failure: Patients with cardiac failure should be treated with caution. In a long-term, placebo controlled study in patients suffering from severe heart failure (NYHA III and IV) of nonischaemic aetiology, amlodipine was associated with increased reports of pulmonary oedema despite no significant difference in the incidence of worsening heart failure as compared to placebo.'

Beta-Blocker Withdrawal: AMCARDIA is not a beta-blocker and therefore gives no protection against the dangers of abrupt beta-blocker withdrawal; any such withdrawal should be by gradual reduction of the dose of beta-blocker.

Patients with Hepatic Failure: Since AMCARDIA is extensively metabolized by the liver and the plasma elimination half-life (t ½) is 56 hours in patients with impaired hepatic function, caution should be exercised when administering AMCARDIA to patients with severe hepatic impairment.

For specialist's use only

Pregnancy and Lactation:

Pregnancy: There are no adequate data on the use of amlodipine in pregnant women. In animal studies; effects on reproduction were found at high dosages. The potential risk for humans is unknown. Accordingly, amlodipine should not be used during pregnancy unless strictly indicated.

Nursing Mothers: It is not known whether amlodipine is excreted in human milk. In the absence of this information, it is recommended that nursing be discontinued while AMCARDIA is administered.

Pediatric Use: Safety and effectiveness of AMCARDIA in children have not been established.

Side effects: In general, treatment with Amlodipine besilate was well-tolerated at doses up to 10 mg daily. Most adverse reactions reported during therapy with Amlodipine besilate were of mild or moderate severity. The most common side effects are headache and edema.

Cardiovascular:

Arrhythmia (including ventricular tachycardia and atrial fibrillation), bradycardia, chest pain, hypotension, peripheral ischemia, syncope, tachycardia, postural dizziness, postural hypotension. **Central and Peripheral Nervous System:** hypoesthesia, paresthesia, tremor, vertigo. **Gastrointestinal:** anorexia, constipation, dyspepsia, dysphagia, diarrhea, flatulence, vomiting, gingival hyperplasia. **General:** Asthenia, back pain, hot flushes, malaise, pain, rigors, weight gain. **Musculoskeletal System:** Arthralgia, arthrosis, muscle cramps, myalgia. **Psychiatric:** sexual dysfunction (male and female), insomnia, nervousness, depression, abnormal dreams, anxiety, depersonalization.

Respiratory System: Dyspnea, epistaxis. **Skin and Appendages:** Pruritus, rash, rash erythematous, rash maculopapular.

Interaction with Other Medicament:

In vitro data in human plasma indicated that AMCARDIA has no effect on the protein binding of drugs tested (digoxin, phenytoin, warfarin, and indomethacin). Amlodipine besilate has been safely administered.

Symptoms and Treatment of Overdose:

If massive overdose should occur, active cardiac and respiratory monitoring should be instituted. Frequent blood pressure measurements are essential. Should hypotension occur, cardiovascular support including elevation of the extremities and the judicious administration of fluids should be initiated. If hypotension remains unresponsive to these conservative measures, administration of vasopressors (such as phenylephrine) should be considered. Intravenous calcium gluconate may help to reverse the effects of calcium entry blockade. As Amlodipine Besilate is highly protein bound, hemodialysis is not likely to be of benefit.

Storage Condition: Store below 30°C in a dry place. Protect from light.

Keep away medicines from the reach of children.

Jauhkan ubat dari kanak-kanak.

Shelf Life: 3yrs

Presentation: 3 or 10 such Blisters of 10s is packed in carton with package insert.

Manufactured by :
UNIQUE PHARMACEUTICAL LABORATORIES
(A Div. of J.B. Chemicals & Pharmaceuticals Ltd.)
Plot No. 215-219, G.I.D.C. Industrial Area, Panoli, Dist. Bharuch, 394116, India.
* Trade Mark

Product Registration Holder and Imported By:

Unimed Sdn Bhd
#53, Jalan Tembaga SD 5/2B, Bandar Sri Damansara 52200,
Kuala Lumpur, Malaysia.
Revised Date: 2/7/2024

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Artist: Vijay